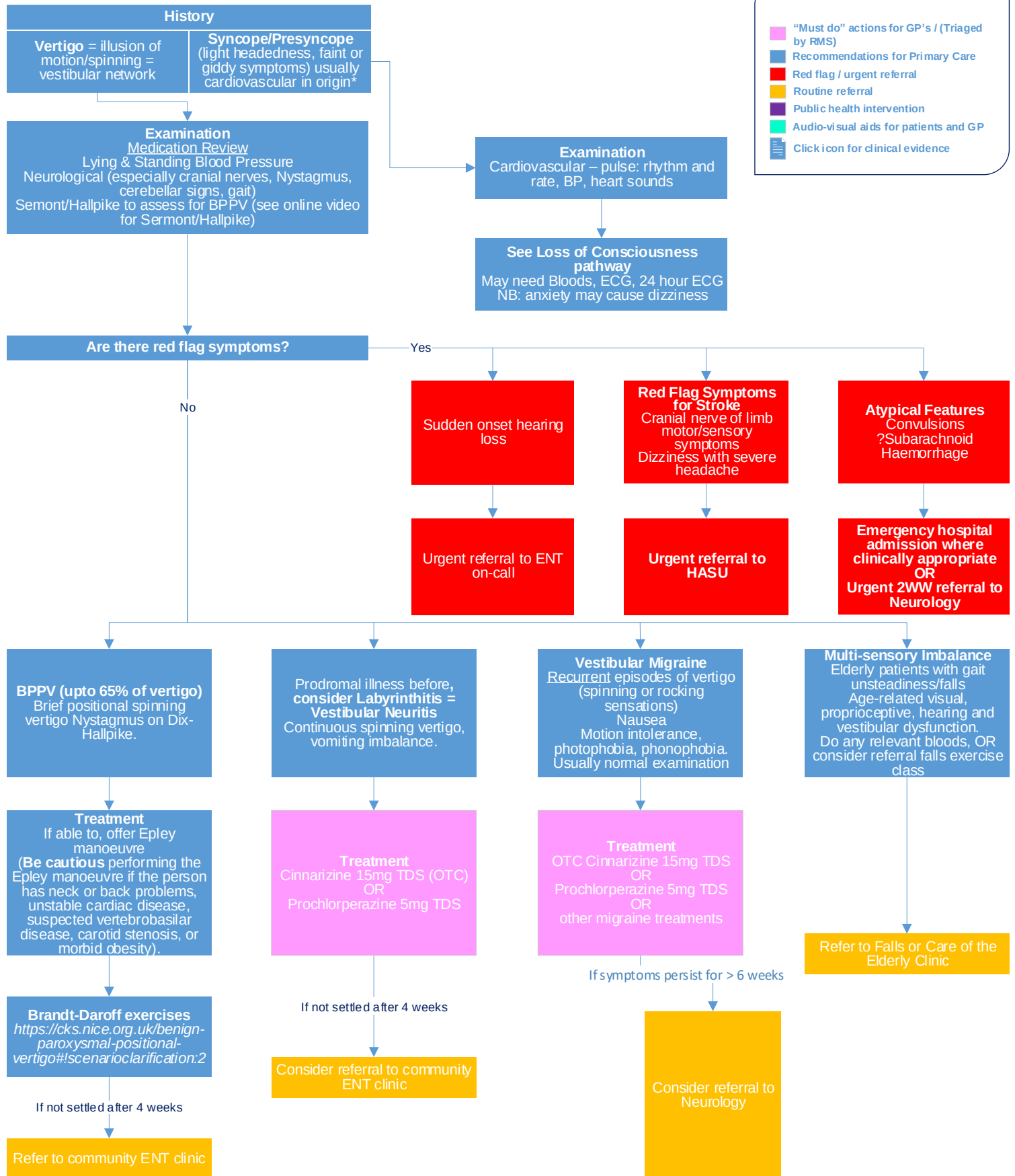


Dizziness Primary Care Protocol

Version 5: February 2019
Review Date: April 2020

Key

- "Must do" actions for GP's / (Triaged by RMS)
- Recommendations for Primary Care
- Red flag / urgent referral
- Routine referral
- Public health intervention
- Audio-visual aids for patients and GP
- Click icon for clinical evidence



General Information Points

AVOID long-term pharmacological use in dizziness or vertigo (e.g. Prochlorperazine (stemetil), Cinnarizine, anticholinergics like scopolamine); there is little evidence or clear effectiveness but they often delay central compensation and create a psychological dependence.

All patients should receive general supportive and reassurance advice as a significant proportion develop secondary avoidance behaviour. They should be advised to mobilise as much as possible as this helps them compensate quicker

NOTE: Consider Advice & Guidance on e-referral to ENT

DVLA states that people with 'liability to sudden and unprovoked or unprecipitated episodes of disabling dizziness' should stop driving and inform the DVLA